

ASCO 2026: REGENERON ONCOLOGY PRESENTATIONS



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C-POST study of adjuvant cemiplimab for high-risk cutaneous squamous cell carcinoma (CSCC): Disease-free survival (DFS) analyses per high-risk criteria and per start time after radiotherapy

Denny Rischin, MD,¹ Sandro V. Porceddu, MD,² Fiona Day, PhD,³ Daniel P. Brungs, MB, BS, MMed, PhD,⁴ Hayden Robert Christie, MB, BS,⁵ James Estes Jackson, MB, BS, PhD,⁶ Brian N. Stein, MD,⁷ Annette M. Lin, MD, PhD,⁸ Yunguo Bernard Su, MD,⁹ Gerard Adams, MB, ChB,¹⁰ Samantha E. Bowyer, MB, BCh,¹¹ Zulfikar Ali Othman, MB, BS,¹² Naoya Yamezaki, MD, PhD,¹³ Paolo Bossi, MD,^{14,15} Annabeth Chellappell, PhD,¹⁶ Rahul Ladwa, MB, ChB, MPhil,¹⁷ Axel Hauschild, MD, PhD,¹⁷ Debra A. G. McIntyre, MS,¹⁸ Jean-Francois Pouliot, PhD,¹⁹ Frank Seebach, MD, PhD,¹⁹ Suk-Young Yoo, PhD,²⁰ Priscilla Goncalves, MD,²¹ Matthew G. Fury, PhD,²²

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BACKGROUND

- Patients with CSCC who are at high risk of disease recurrence after surgery and adjuvant RT need effective adjuvant systemic treatment options¹
- Cemiplimab is approved in many countries for use across multiple solid tumor types, including in adult patients with locally advanced or metastatic CSCC who are not candidates for curative surgery or RT; cemiplimab is also approved in multiple countries for the adjuvant treatment of adult patients with CSCC at high risk of recurrence after surgery and RT^{2,3}
- The C-POST trial (NCT03969004) evaluated cemiplimab as an adjuvant therapy for the treatment of patients at high risk of CSCC recurrence after surgery and RT⁴
 - At primary analysis, DFS was superior with adjuvant cemiplimab vs placebo (HR, 0.32; $P < 0.0001$); safety profile was consistent with the known safety of cemiplimab in advanced solid malignancies⁵
 - Results from the study supported the approval of adjuvant cemiplimab in this patient population^{2,3}
- Additional analyses were conducted to evaluate the effect of each high-risk criterion and the interval between completion of adjuvant RT and study randomization on clinical outcomes

OBJECTIVE

- With approximately 6 months of additional follow-up (data cutoff: April 7, 2025) in C-POST, we present the exploratory analyses of DFS based on the following factors:
 - High risk criteria
 - Interval between completion of adjuvant RT and study randomization

METHODS

- Patients were randomized 1:1 (N=415) to adjuvant cemiplimab or placebo
 - Patients received cemiplimab 350 mg or placebo Q3W for approximately 12 weeks, followed by cemiplimab 700 mg or placebo Q6W for 36 weeks for a total planned treatment duration of 48 weeks (4 cycles; cycle length = 12 weeks)
- All patients had high-risk CSCC defined by nodal and/or non-nodal features (Figure 1)

Figure 1. Nodal and non-nodal high-risk criteria

Nodal disease	In-transit metastases	Perineural invasion	T4 lesions	Recurrent CSCC
ECE with ≥ 1 node ≥ 20 mm OR ≥ 3 nodes regardless of ECE	Skin or subcutaneous metastases > 20 mm from the primary lesion but not beyond the regional nodal basin	Clinical and/or radiologic involvement of named nerves	Invasion of cortical bone or skull base	CSCC that arises within the area of previously resected tumor, plus ≥ 1 additional feature: $\geq N2b$ disease associated with the recurrent lesion - Nominal $\geq T3$ - Poorly differentiated histology and recurrent lesion ≥ 20 mm in diameter

- Tumors had to meet ≥ 1 high-risk criteria
- Study randomization occurred within 2-11 weeks after completion of adjuvant RT
- DFS exploratory analyses were performed based on the following factors:
 - High risk criteria presented in Figure 1
 - Interval between completion of adjuvant RT and study randomization (2-6 weeks vs > 6 weeks)
- The treatment effect on DFS was assessed in a stratified Cox proportional hazard model as the interaction between each criterion and treatment



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Poster

Presented at the ASCO 2025 Annual Meeting,
Chicago, IL, USA;
May 29–June 2, 2025

Presenting author: Denny Rischin

CONCLUSIONS

POSTER 6083

- Long-term follow-up (median, 30 months) of the phase 3 C-POST study showed continued DFS benefit with adjuvant cemiplimab vs placebo (HR, 0.35) in patients with high-risk CSCC
- In the exploratory analyses, adjuvant cemiplimab demonstrated a DFS benefit vs placebo across all high-risk categories
- A consistent benefit of cemiplimab was observed regardless of whether the interval between adjuvant RT completion and randomization was 2-6 weeks or > 6 weeks

RESULTS

- As of April 7, 2025, median duration of follow-up was 30 months
- Among 415 patients (cemiplimab, n=209; placebo, n=206) randomized, the DFS HR was 0.35 (95% CI, 0.23-0.55)

DFS per high-risk criteria

- The most common high-risk criterion was nodal disease with ECE, present in 48.4% of patients
- DFS was improved with cemiplimab vs placebo across all high-risk categories, including both nodal and non-nodal criteria (Figure 2)

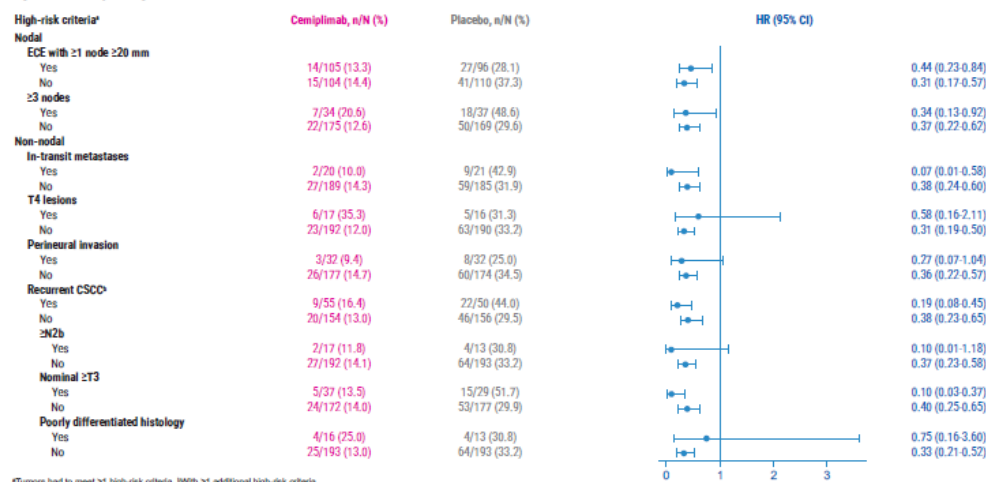
DFS per the interval between completion of adjuvant RT and study randomization

- The median time from completion of adjuvant RT to study randomization was 5.65 weeks
- The interval between adjuvant RT completion and study randomization did not impact the DFS benefit of cemiplimab vs placebo (Table 1)

Safety

- At 30-month follow-up, the overall incidence of any-grade TEAEs was comparable in the cemiplimab and placebo arms, consistent with the results from primary analysis (Supplementary Table 1)

Figure 2. DFS per high-risk criteria



*Tumors had to meet ≥ 1 high-risk criteria. *With ≥ 1 additional high-risk criteria.

Table 1. DFS per start time after adjuvant RT

Parameter	Cemiplimab, n/N (%)	Placebo, n/N (%)	HR (95% CI)
Interval between prior RT and study randomization			
2-6 weeks	17/117 (14.5)	37/112 (33.0)	0.39 (0.22-0.70)
> 6 weeks	12/91 (13.2)	31/93 (33.3)	0.36 (0.19-0.71)

ABBREVIATIONS

CI, confidence interval; CSCC, cutaneous squamous cell carcinoma; DFS, disease-free survival; ECE, extracapsular extension; HR, hazard ratio; Q3W, every 3 weeks; Q6W, every 6 weeks; RT, radiotherapy; T4, tumor stage 4; T4C, tumor stage 4, T4C, treatment-emergent adverse event.

FUNDING

This study (NCT03969004) was funded by Regeneron Pharmaceuticals, Inc., and Sanofi.

Supplementary data, references, author disclosures, and acknowledgments are available in the Supplementary Information.

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C-POST study of adjuvant cemiplimab for high-risk cutaneous squamous cell carcinoma (CSCC): disease-free survival (DFS) analyses per high-risk criteria and per start time after radiotherapy

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SUPPLEMENTARY

SUPPLEMENTARY RESULTS

Supplementary Table 1. Safety summary

	Cemiplimab (n=205)		Placebo (n=204)	
TEAEs, regardless of attribution, n (%)	Any grade	Grade ≥3	Any grade	Grade ≥3
Any	191 (93.2)	52 (25.4)	185 (90.7)	33 (16.2)
Serious	38 (18.5)	33 (16.1)	21 (10.3)	16 (7.8)
Leading to study treatment discontinuation	20 (9.8)	16 (7.8)	4 (2.0)	3 (1.5)
Leading to death	2 (1.0)	2 (1.0)	3 (1.5)	3 (1.5)
Treatment-related TEAEs, n (%)	Any grade	Grade ≥3	Any grade	Grade ≥3
Any	137 (66.8)	21 (10.2)	98 (48.0)	1 (0.5)
Immune-mediated AEs, n (%)	Any grade	Grade ≥3	Any grade	Grade ≥3
Any	52 (25.4)	15 (7.3)	17 (8.3)	0

AE, adverse event; TEAE, treatment-emergent adverse event.

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1. Porceddu SV, et al. *J Clin Oncol*. 2018;36:1275-1283.
2. LIBTAYO (cemiplimab-rwc) injection. Prescribing information. Regeneron Pharmaceuticals, Inc. Accessed April 10, 2026. Available at: https://www.regeneron.com/downloads/libtayo_fpi.pdf
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4. ClinicalTrials.gov. Study of adjuvant cemiplimab versus placebo after surgery and radiation therapy in patients with high risk cutaneous squamous cell carcinoma. Accessed April 10, 2026. Available at: <https://clinicaltrials.gov/study/NCT03969004>
5. Rischin D, et al. *N Engl J Med*. 2025;393:774-785.

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Presenting Author: Danny Rischin

Presented at the ASCO 2026 Annual Meeting:
Chicago, IL, USA;
May 29–June 2, 2026

DISCLOSURES

1. Danny Rischin reports institutional research funding from ALX Oncology, AstraZeneca, Bicara Therapeutics, Merck, and Regeneron Pharmaceuticals, Inc., and uncompensated relationships with GlaxoSmithKline, Merck, Bicara Therapeutics, Medison Pharma, and Regeneron Pharmaceuticals, Inc.
2. Sandro V. Porceddu reports leadership role in ICON Cancer Care Australia; advisory role with Regeneron Pharmaceuticals, Inc.; and intellectual property interests with UpToDate.
3. Fiona Day reports speakers contract with AstraZeneca Australia; institutional research support from AstraZeneca Australia and Bristol Myers Squibb; participation in an advisory board for Medison; and travel costs provided by Merck.
4. Daniel P. Brungs reports honoraria from Merck Sharp & Dohme and consulting/advisory role with AstraZeneca.
5. Hayden Robert Christie reports honoraria from Bayer; and travel, accommodation, and/or other expenses from Merck Sharp & Dohme.
6. James Estes Jackson is a shareholder of ICON Cancer Centre.
7. Brian N. Stein is a shareholder of ICON Cancer Centre PTY LTD.
8. Annette M. Lim reports uncompensated consultancy for Pfizer, Eli Lilly, and Merck Healthcare; support for an investigator-initiated trial and translational research support from Regeneron Pharmaceuticals, Inc.; received by the institution; conference registration from Regeneron Pharmaceuticals, Inc.; and salary support from a Peter MacCallum Cancer Centre Discovery Partner Fellowship.
9. Yunguo Bernard Su and Gerard Adams report no conflicts of interest.
10. Samantha E. Buey reports payments for speaking roles from Merck Sharp & Dohme, AstraZeneca, Bristol Myers Squibb, Medison, and Boehringer Ingelheim; travel support from AstraZeneca, Janssen Oncology, Boehringer Ingelheim, and Merck Sharp & Dohme; and payment for participation in an advisory board from Merck Sharp & Dohme, Roche, Boehringer Ingelheim, Janssen, and Medison.
11. Zulfar Ali Qazi reports speakers bureau with AstraZeneca and Bristol Myers Squibb; and travel, accommodation, and expenses from AstraZeneca, Merck Sharp & Dohme, and Bristol Myers Squibb.
12. Naoya Yamazaki reports speakers bureau with Ono, Bristol Myers Squibb, Merck Sharp & Dohme, and Novartis; and research funding from Bristol Myers Squibb, Merck Sharp & Dohme, Novartis, Takara Bio, Regeneron Pharmaceuticals, Inc., and HUYA Bioscience International.
13. Paolo Bressi reports personal fees from Merck, AstraZeneca, Pfizer, and Bicara Therapeutics; grants and personal fees from Merck Sharp & Dohme and Regeneron Pharmaceuticals, Inc.; and grants from Boehringer.
14. Anamath Challeppalli reports consulting/advisory roles with Sanofi, Regeneron Pharmaceuticals, Inc., and Merck; institutional research funding from Regeneron Pharmaceuticals, Inc.; and travel, accommodation, and expenses from Ipsen and Janssen.
15. Rahul Ladava reports honoraria from Merck Sharp & Dohme and Sanofi; consulting/advisory roles with Merck Sharp & Dohme and La Roche-Posay; research funding from Merck Sharp & Dohme; and travel, accommodation, and expenses from Sanofi and Merck Sharp & Dohme.
16. Axel Hauschild reports honoraria from Bristol Myers Squibb, Merck Serono, Merck Sharp & Dohme, Novartis, Roche, Philogen, Regeneron Pharmaceuticals, Inc., Sanofi/Aventis, Immunocore, Regimune, Agenzia, Incepta, Curevac, IQ Biotech, HeadCare GmbH, Syntex Diagnostics, Pharma Fabre, AstraZeneca GmbH, Xerthera, and Sun Pharma; consulting/advisory roles with Amgen, Bristol Myers Squibb, Merck Serono, Merck Sharp & Dohme, Novartis, Roche, Philogen, Regeneron Pharmaceuticals, Inc., Sanofi/Aventis, Immunocore, Regimune, and IQ Biotech; research funding from Amgen, Bristol Myers Squibb, Merck Serono, Merck Sharp & Dohme, Novartis, Roche, and Regeneron Pharmaceuticals, Inc.; and travel, accommodation, and expenses from Amgen, Bristol Myers Squibb, Merck Serono, Merck Sharp & Dohme, Novartis, Roche, and Sanofi/Aventis.
17. Debra A. G. McIntyre, Jean-Francois Pouliot, Frank Seebach, Suk-Young Yoo, Priscilla Goncalves, and Matthew G. Fury are employees and shareholders of Regeneron Pharmaceuticals, Inc.

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Background

- Patients with CSCC who are at high risk of disease recurrence after surgery and adjuvant RT need effective adjuvant systemic treatment options¹
- Cemiplimab is approved in many countries for use across multiple solid tumor types, including in adult patients with locally advanced or metastatic CSCC who are not candidates for curative surgery or RT; cemiplimab is also approved in multiple countries for the adjuvant treatment of adult patients with CSCC at high risk of recurrence after surgery and RT^{2,3}
- The C-POST trial (NCT03969004) evaluated cemiplimab as an adjuvant therapy for the treatment of patients at high risk of CSCC recurrence after surgery and RT⁴
 - At primary analysis, DFS was superior with adjuvant cemiplimab vs placebo (HR, 0.32; P<0.0001); safety profile was consistent with the known safety of cemiplimab in advanced solid malignancies⁵
 - Results from the study supported the approval of adjuvant cemiplimab in this patient population^{2,3}
- Additional analyses were conducted to evaluate the effect of each high-risk criterion and the interval between completion of adjuvant RT and study randomization on clinical outcomes

CSCC, cutaneous squamous cell carcinoma; DFS, disease-free survival; HR, hazard ratio; RT, radiotherapy.

1. Porceddu SV, et al. *J Clin Oncol*. 2018;36:1275-1283. 2. LIBTAYO (cemiplimab-rwlc) injection. Prescribing information. Regeneron Pharmaceuticals, Inc. Accessed April 10, 2026. Available at: https://www.regeneron.com/downloads/libtayo_fpi.pdf. 3. LIBTAYO Summary of Product Characteristics. Regeneron Pharmaceuticals, Inc. Accessed April 10, 2026. Available at: https://www.ema.europa.eu/en/documents/product-information/libtayo-epar-product-information_en.pdf. 4. ClinicalTrials.gov. Study of adjuvant cemiplimab versus placebo after surgery and radiation therapy in patients with high risk cutaneous squamous cell carcinoma. Accessed April 10, 2026. Available at: <https://clinicaltrials.gov/study/NCT03969004>. 5. Rischin D, et al. *N Engl J Med*. 2025;393:774-785.

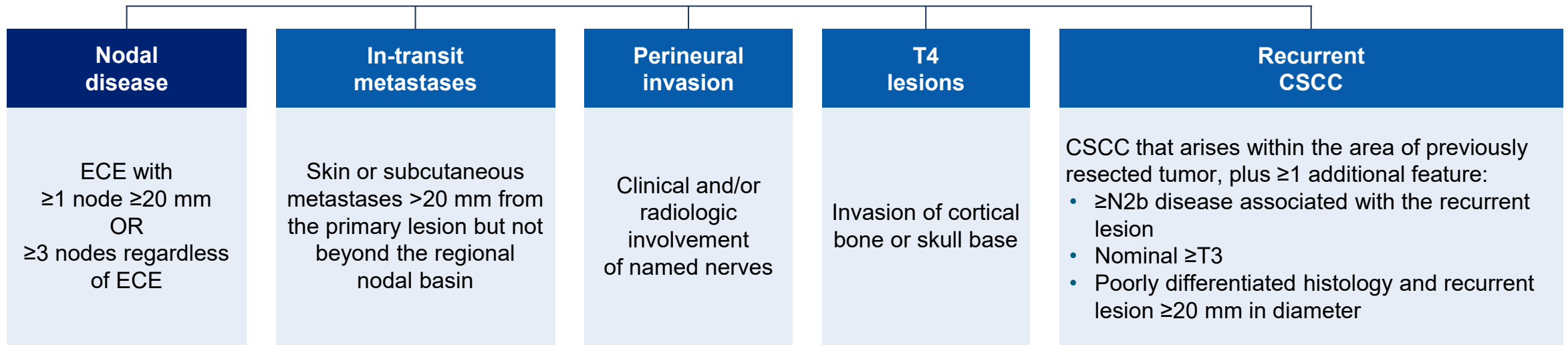
Objective

- With approximately 6 months of additional follow-up (data cutoff: April 7, 2025) in C-POST, we present the exploratory analyses of DFS based on the following factors:
 - High-risk criteria
 - Interval between completion of adjuvant RT and study randomization

Methods

- Patients were randomized 1:1 (N=415) to adjuvant cemiplimab or placebo
 - Patients received cemiplimab 350 mg or placebo Q3W for approximately 12 weeks, followed by cemiplimab 700 mg or placebo Q6W for 36 weeks for a total planned treatment duration of 48 weeks (4 cycles; cycle length = 12 weeks)
- All patients had high-risk CSCC defined by nodal and/or non-nodal features (**Figure 1**)
- Tumors had to meet ≥ 1 high-risk criteria

Figure 1. Nodal and non-nodal high-risk criteria



Methods

- Study randomization occurred within 2-11 weeks after completion of adjuvant RT
- DFS exploratory analyses were performed based on the following factors:
 - High-risk criteria presented in **Figure 1**
 - Interval between completion of adjuvant RT and study randomization (2-6 weeks vs >6 weeks)
- The treatment effect on DFS was assessed in a stratified Cox proportional hazard model as the interaction between each criterion and treatment

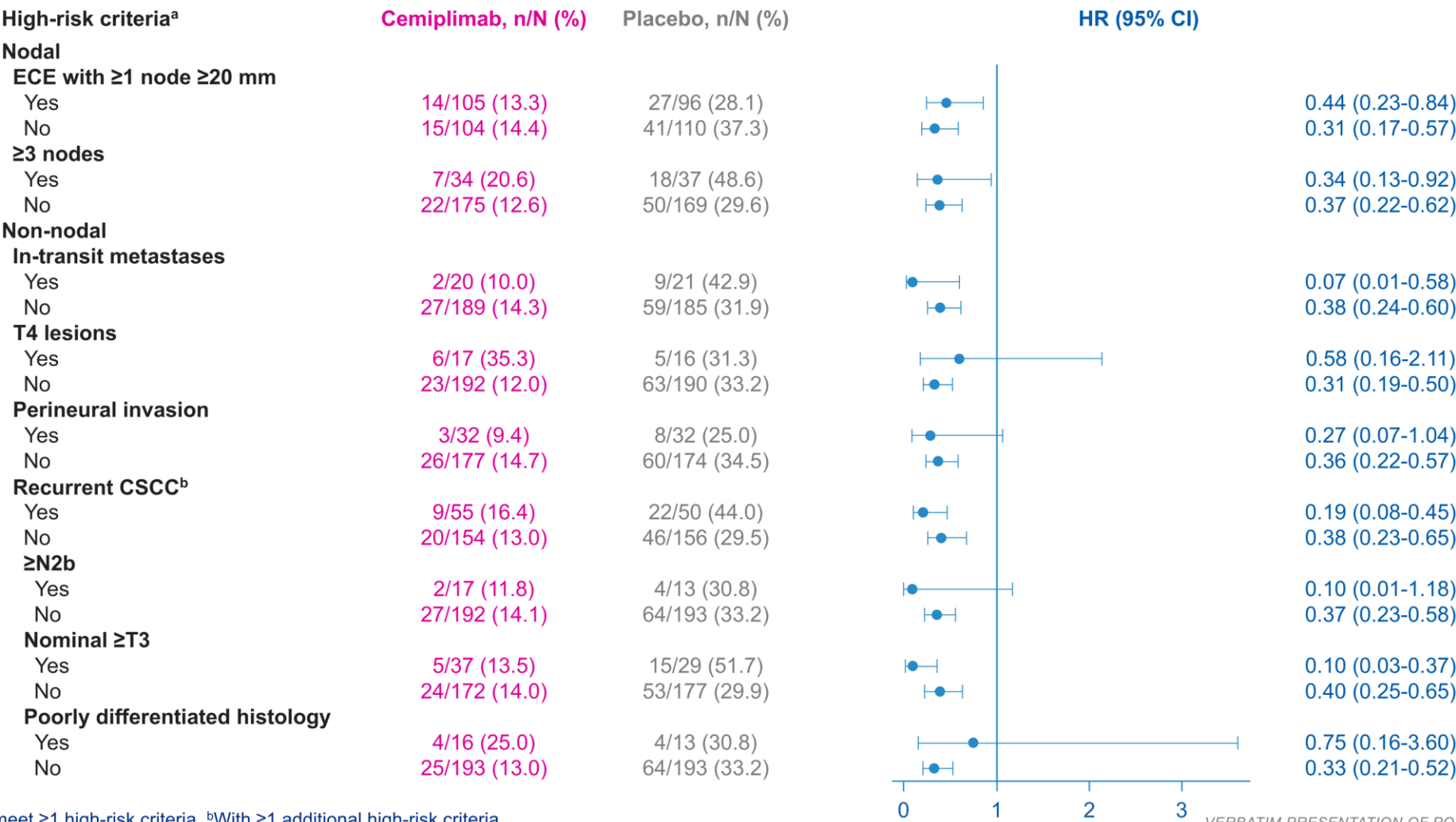
Results

- As of April 7, 2025, median duration of follow-up was 30 months
- Among 415 patients (cemiplimab, n=209; placebo, n=206) randomized, the DFS HR was 0.35 (95% CI, 0.23-0.55)

DFS per high-risk criteria

- The most common high-risk criterion was nodal disease with ECE, present in 48.4% of patients
- DFS was improved with cemiplimab vs placebo across all high-risk categories, including both nodal and non-nodal criteria (**Figure 2**)

Figure 2: DFS per high-risk criteria



^aTumors had to meet ≥1 high-risk criteria. ^bWith ≥1 additional high-risk criteria.
CI, confidence interval; CSCC, cutaneous squamous cell carcinoma; DFS, disease-free survival; ECE, extracapsular extension; HR, hazard ratio; T3, tumor stage 3; T4, tumor stage 4.

Results: DFS per the interval between completion of adjuvant RT and study randomization

- The median time from completion of adjuvant RT to study randomization was 5.65 weeks
- The interval between adjuvant RT completion and study randomization did not impact the DFS benefit of cemiplimab vs placebo (**Table 1**)

Table 1. DFS per start time after adjuvant RT

Parameter	Cemiplimab, n/N (%)	Placebo, n/N (%)	HR (95% CI)
Interval between prior RT and study randomization			
2-6 weeks	17/117 (14.5)	37/112 (33.0)	0.39 (0.22-0.70)
>6 weeks	12/91 (13.2)	31/93 (33.3)	0.36 (0.19-0.71)

Results: Safety

- At 30-month follow-up, the overall incidence of any-grade TEAEs was comparable in the cemiplimab and placebo arms, consistent with the results from primary analysis
(**Supplementary Table 1**)

Supplementary Table 1. Safety summary

	Cemiplimab (n=205)		Placebo (n=204)	
TEAEs, regardless of attribution, n (%)	Any grade	Grade ≥3	Any grade	Grade ≥3
Any	191 (93.2)	52 (25.4)	185 (90.7)	33 (16.2)
Serious	38 (18.5)	33 (16.1)	21 (10.3)	16 (7.8)
Leading to study treatment discontinuation	20 (9.8)	16 (7.8)	4 (2.0)	3 (1.5)
Leading to death	2 (1.0)	2 (1.0)	3 (1.5)	3 (1.5)
Treatment-related TEAEs, n (%)	Any grade	Grade ≥3	Any grade	Grade ≥3
Any	137 (66.8)	21 (10.2)	98 (48.0)	1 (0.5)
Immune-mediated AEs, n (%)	Any grade	Grade ≥3	Any grade	Grade ≥3
Any	52 (25.4)	15 (7.3)	17 (8.3)	0

Conclusions

- Long-term follow-up (median, 30 months) of the phase 3 C-POST study showed continued DFS benefit with adjuvant cemiplimab vs placebo (HR, 0.35) in patients with high-risk CSCC
- In the exploratory analyses, adjuvant cemiplimab demonstrated a DFS benefit vs placebo across all high-risk categories
- A consistent benefit of cemiplimab was observed regardless of whether the interval between adjuvant RT completion and randomization was 2-6 weeks or >6 weeks

Acknowledgments

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- Writing assistance was provided by Saumya Radhakrishnan, BTech, of MEDiSTRAVA, funded by Regeneron Pharmaceuticals, Inc.

Disclosures

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- **Sandro V. Porceddu** reports leadership role in ICON Cancer Care Australia; advisory role with Regeneron Pharmaceuticals, Inc.; and intellectual property interests with UpToDate
- **Fiona Day** reports speakers contract with AstraZeneca Australia; institutional research support from AstraZeneca Australia and Bristol Myers Squibb; participation in an advisory board for Medison; and travel costs provided by Merck
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- **Annette M. Lim** reports uncompensated consultancy for Pfizer, Eisai, Lilly, and Merck Healthcare; support for an investigator-initiated trial and translational research support from Regeneron Pharmaceuticals, Inc., received by the institution; conference registration from Regeneron Pharmaceuticals, Inc.; and salary support from a Peter MacCallum Cancer Centre Discovery Partner Fellowship

Disclosures

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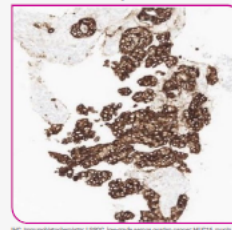
A Phase 1/2 study of ubamatamab (REGN4018), a MUC16×CD3 bispecific antibody, administered alone or in combination with cemiplimab in a low-grade serous ovarian cancer cohort: Trial in progress

Els Van Nieuwenhuysen,¹ Judith Michels,² Hee Seung Kim,³ Victor Moreno,⁴ Nicoletta Colombo,⁵ Petronella B. Ottavanger,⁶ Rolsin E. O'Carroll,⁷ Ruth Perets,⁸ Anna K. L. Reyniers,⁹ Suk-Young Yoo,¹⁰ Bin Wang,¹⁰ Juriaan Brouwer-Visser,¹⁰ Angela K. Green,¹⁰ Nicole Costigan,¹⁰ Mary Peterman,¹⁰ Brigid Barnes,¹⁰ Israel Lowy,¹⁰ Elizabeth Miller,¹⁰ Gloria S. Huang,¹⁰ Thomas S. Uldrick¹⁰

BACKGROUND

- There is a high unmet need for novel treatment options for patients with low-grade serous ovarian cancer (LGSOC) that are efficacious and better tolerated¹
- Ubamatamab (REGN4018) is a mucin 16 × cluster of differentiation 3 (MUC16×CD3) bispecific antibody that bridges MUC16 positive tumor cells and CD3, expressed on T cells, through a major histocompatibility complex independent mechanism, promoting cytotoxicity²
- Sarilumab, an anti-interleukin 6 receptor α subunit monoclonal antibody, is currently approved in multiple indications and has demonstrated promising mitigation of ubamatamab associated cytokine release syndrome^{3,4}
- In Phase 1 of this first-in-human study (NCT03564340), ubamatamab demonstrated an acceptable safety profile and durable clinical activity in patients with recurrent ovarian cancer, including LGSOC (n=19), which has particularly high MUC16 expression on tumor cells (Figure 1)^{5,6}

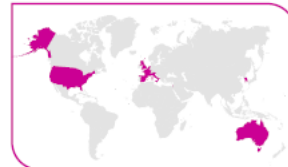
FIGURE 1. MUC16 IHC in LGSOC; 100% of cells express MUC16



CONCLUSIONS

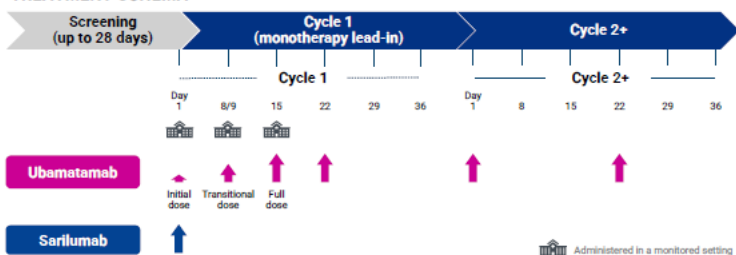
- After ubamatamab step-up dosing with sarilumab prophylaxis, 50 patients will be treated with ubamatamab
- If ≥14/50 patients have a response, the cohort may be expanded to 100 patients
- As of April 20, 2026, 24 patients with LGSOC have initiated ubamatamab treatment, with 51 sites currently open for enrollment across 10 countries (Figure 2)
- Insights into the efficacy and safety of ubamatamab in patients with LGSOC may further validate MUC16 as a novel therapeutic target for LGSOC

FIGURE 2. Enrollment sites



The aim of this disease-specific expansion cohort of an open-label, Phase 1/2, first-in-human study (NCT03564340) is to evaluate the safety and clinical activity of ubamatamab monotherapy in patients with LGSOC

TREATMENT SCHEMA



ENDPOINTS

Primary

- Objective response rate per Response Evaluation Criteria in Solid Tumors 1.1

Key secondary

- Best overall response
- Duration of response
- Disease control rate
- Complete response rate
- Progression-free survival
- Safety
- Pharmacokinetics
- Immunogenicity
- Patient-reported outcomes

STATISTICAL ANALYSIS PLAN

- Initially, 50 patients will be enrolled, as determined by Simon's 2-stage design
- The cohort may be expanded to 100 patients if there are ≥14/50 responses (28%)

ELIGIBILITY CRITERIA

TABLE 1. Key inclusion criteria
Aged ≥18 years
Histologically or cytologically confirmed diagnosis of LGSOC
Received prior platinum-containing therapy*
Progression or recurrence that is documented radiographically, on or after the most recent line received, with at least one measurable lesion
No standard therapy likely to convey clinical benefit
ECOG PS of 0 or 1
Adequate organ and bone marrow function

*There is no maximum number of prior therapies for patients in the LGSOC-specific expansion cohort.
ECOG PS, Eastern Cooperative Oncology Group performance score; LGSOC, low-grade serous ovarian cancer.

TABLE 2. Key exclusion criteria
Prior treatment with a MUC16-targeted therapy
Prior treatment with anti-PD-1/PD-L1 therapy (<5 half-lives of first dose) or CAR T therapy (<30 days of first dose)
Prior immune-mediated toxicities requiring permanent discontinuation of immunotherapy
Untreated or active primary brain tumor, CNS metastases, or spinal cord compression
History of clinically significant cardiac disease within 6 months before screening
Active, uncontrolled infection or immunodeficiency
LVEF <50%, as measured by echocardiogram at baseline

CAR T, chimeric antigen receptor T cell; CNS, central nervous system; LVEF, left ventricular ejection fraction; MUC16, mucin 16; PD-1, programmed cell death-1; PD-L1, programmed cell death-ligand 1.



Solid Tumors
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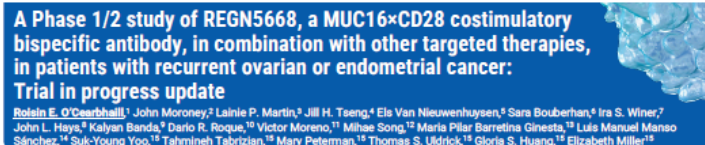
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- Platinum-resistant ovarian cancer (OC) and endometrial cancer (EC) that recurs after anti-programmed cell death-1 (PD-1) therapy are associated with poor prognoses,^{1,2} presenting an unmet need for novel therapies that can improve long-term outcomes for patients
- Mucin 16 (MUC16), a cell surface glycoprotein highly expressed on OC³ and EC⁴ tumor cells, has been identified as a potential therapeutic target
- RGX5668⁵ and ubamatamab⁶ are immunoglobulin G4-based bispecific antibodies that bridge MUC16 positive tumor cells to T-cell-expressed cluster of differentiation (CD) 28 and CD3, respectively, to stimulate T cell cytotoxicity (Figure 1)
- The monoclonal antibodies cemiplimab (anti-PD-1)⁷ and nivolumab (anti-lymphocyte activation gene 3)⁸ target inhibitory T cell immune checkpoints
- Results from Module 1P dose escalation of this study demonstrated early clinical activity and an acceptable RGX5668 safety profile in patients with platinum-resistant recurrent OC⁹

CD28 ligand of Urethane-binding protein (MUC16-CD28)

Signal 1

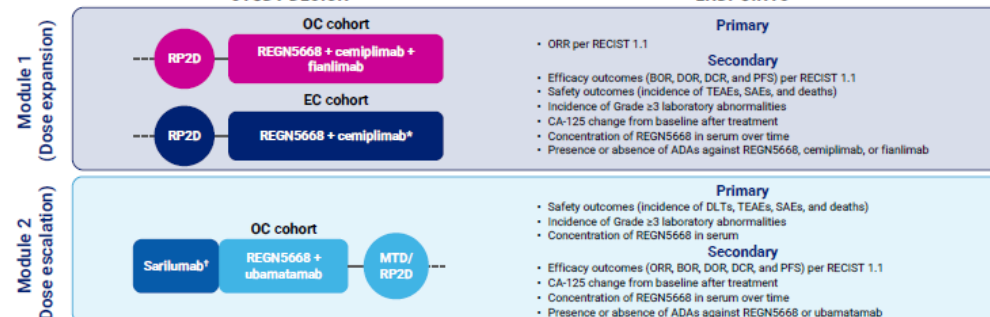
Signal 2

TPS5639

- This study will evaluate REGN5668 + cemiplimab + fianlimab or REGN5668 + ubamatamab in patients with recurrent OC or EC
- As of May 6, 2026, in 22 sites across 4 countries (Figure 2), Module 1 (dose expansion) has enrolled 14 patients (10 OC, 4 EC) with Module 2 (dose escalation) having progressed through multiple dose levels

This first-in-human, Phase 1/2 study (NCT04590326) will assess the safety, tolerability, pharmacokinetics, and antitumor activity of REGN5668 + cemiplimab + flanimab (Module 1) or REGN5668 + ubamatamab (Module 2) in patients with platinum-resistant recurrent OC or EC

ENDPOINTS



The additional expansion cohort to evaluate RIBONSIM + amgivalin + faricimab in patients with HC may be initiated, following the safety results for this combination in patients with DC and as ongoing as the tolerability observed in patients with HC treated with RIBONSIM + amgivalin. Faricimab is an anti-VEGF monoclonal antibody that is being evaluated in phase 3 studies for idiopathic intracranial hypertension (IIH), wet age-related macular degeneration (AMD), and wet diabetic macular edema (DME). ADA, anti-drug antibody; DC, ductal carcinoma; DLT, dose-limiting toxicity; DOR, duration of response; EC, endometrial cancer; E-SR, endometrioid serous; MTC, metastatic thyroid cancer; OC, ovarian cancer; ORR, objective response rate; RIBONSIM, RIBONSIM 100 mg/0.5 mg/0.5 mg intravitreal injection; SR, serous; TSS, treatment-related serious adverse event.

Table 1. Key inclusion criteria

OC cohort	EC cohort
Histologically or cytologically confirmed advanced epithelial OC (except carcinosarcoma), primary peritoneal or fallopian tube cancer Relapse or disease progression after or after most recent line of therapy Treated with ≥1 prior line of platinum-based systemic therapy Serum CA-125 level ≥2× ULN Expansion cohorts only: ≥1 lesion that is measurable by RECIST 1.1	Histologically confirmed EC Disease progression or recurrence after anti-PO therapy and platinum-based chemotherapy ≥25% MUC16-positive tumor cells ≥4 prior lines of systemic therapy* ECOG PS 0 or 1 Life expectancy ≥3 months

*Does not include patients adjusted therapy administered ≥ 12 months before first treatment in the recurrent setting or hormonal therapy as a first.

[illegible]

Table 2. Key exclusion criteria

All cohorts
Current or recent treatment with an investigational agent, systemic biologic therapy, or anticancer immunotherapy
Another malignancy within the past 5 years that is progressing, requires active treatment, or has a high likelihood of recurrence
Prior treatment with a MUC16-targeted therapy
Untreated or active CNS malignancies or metastases
Any condition requiring continuous corticosteroid therapy within 1 week prior to first study drug dose
Ongoing or recent (>5 years) evidence of significant autoimmune disease that requires/treated with systemic immunosuppressive treatments
History of clinically significant cardiovascular disease
OC progression within 12 weeks of last systemic therapy

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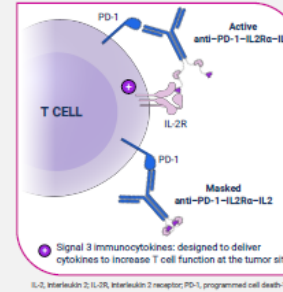
Phase 1/2 dose escalation/expansion study of REGN10597 (anti-PD-1-IL2Rα-IL2) in patients with advanced solid tumors

Kyriakos P. Papadopoulos,¹ Nehal J. Lakhani,² Shaker A. Khan,³ Rodabe H. Amerin,⁴ Tracy L. Ross,⁵ Randy F. Swain,⁶ Anthony B. El-Khouk,⁷ Chiesara Oliver,⁸ Michael S. Havel,⁹ Adil L. Daud,¹⁰ Roderigo I. Rodriguez-Rivera,¹¹ Raika Brown,¹² Teresa Montague,¹³ Shaheda Jami,¹⁴ Elizabeth Miller,¹⁵ Israel Lowy,¹⁶ Melissa Mathias,¹⁷ David Chen,¹⁸ Adi Dab¹⁹

BACKGROUND

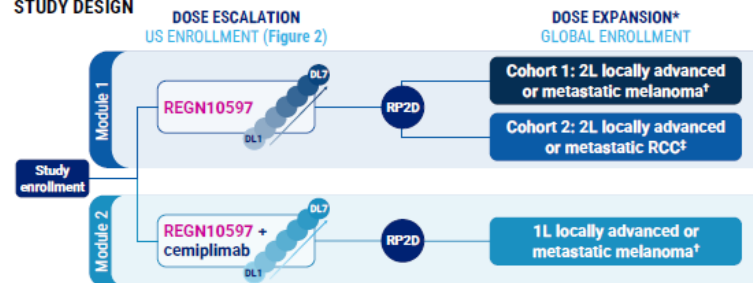
- Interleukin 2 (IL-2) promotes lymphocyte proliferation and differentiation during the anticancer immunomodulatory response¹
- Aldesleukin, an approved recombinant, high-dose IL-2 therapy, has shown complete and durable responses in some patients^{1,2}
 - However, use of high-dose IL-2 is limited by severe toxicity, including vascular leak syndrome and pulmonary edema²⁻⁴
- Despite clinical advances with checkpoint inhibitor therapies (e.g., anti-programmed cell death-1 [PD-1]), advanced solid tumors can fail to show durable responses^{1,5}
- REGN10597 is an antibody-cytokine fusion protein comprising a human anti-PD-1 antibody fused with a receptor-masked cytokine, IL2Rα-IL2⁶ (Figure 1)
- IL-2 receptor (IL-2R) α engagement is a crucial mechanism of IL-2-mediated antitumor activity⁷
 - PD-1-IL2Rα-IL2 exhibits enhanced PD-1 targeting-dependent IL-2 antitumor activity and mitigates IL-2-mediated toxicity⁸
- REGN10597 has demonstrated antitumor efficacy, enhanced specificity, and reduced toxicity versus recombinant high-dose IL-2 in preclinical mouse models⁹

FIGURE 1. REGN10597 mechanism of action



Brilliance: Open-label, Phase 1/2, dose escalation/expansion, first-in-human, multicenter study evaluating safety, tolerability, pharmacokinetics, and preliminary antitumor activity of REGN10597 alone or with cemiplimab in patients with advanced solid tumors (NCT06413680)

STUDY DESIGN



*Dose expansion cohorts were chosen to explore REGN10597 in historically IL-2-sensitive tumor types¹⁴ (Excluding uveal, scleral, or mucosal melanoma; IClear cell RCC; 1L, first line; 2L, second line; AE, adverse event; DL, dose level; IL-2, Interleukin 2; RCC, renal cell carcinoma; RECIST 1.1, Response Evaluation Criteria in Solid Tumors version 1.1; RP2D, recommended Phase 2 dose).

ELIGIBILITY CRITERIA

Key inclusion criteria	
Dose escalation	Dose expansion
Aged ≥18 years	Aged ≥18 years
Histologically or cytologically confirmed diagnosis of solid malignancy	Histologically or cytologically confirmed diagnosis of locally advanced or metastatic malignancy:
Locally advanced or metastatic disease	Module 1 – anti-PD-(L)1 progressed:
Confirmed disease progression on standard-of-care therapy	• 2L melanoma* (Cohort 1)
Archival tissue sample (if available)	• 2L RCC* (Cohort 2)
	Module 2 – 1L melanoma*
	Fresh pretreatment biopsy during screening

*Excluding uveal, scleral, or mucosal melanoma; IClear cell RCC; 1L, first line; 2L, second line; PD-(L)1, programmed cell death-(ligand) 1; RCC, renal cell carcinoma.



Solid Tumors

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APPROVALS

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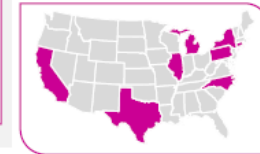
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CONCLUSIONS TPS2675

- PD-1-IL2Rα-IL2 engagement further enhances IL-2-mediated antitumor activity through PD-1-dependent IL-2 signaling with the additional benefit of IL-2 mediated toxicity mitigation
- REGN10597, an IL2Rα-IL2 antibody-cytokine fusion protein, is under investigation in patients with advanced solid tumors
- As of May 15, 2024, 41 patients have been enrolled in the dose escalation phase



FIGURE 2. Dose escalation enrollment sites



ENDPOINTS

PRIMARY

Dose escalation

- Incidence of dose-limiting toxicities, treatment-emergent AEs (including those leading to treatment discontinuation or death), and serious AEs
- Number of patients with Grade ≥3 laboratory abnormalities

Dose expansion

- Objective response rate per RECIST 1.1 by investigator assessment

KEY SECONDARY

- Additional efficacy measures per RECIST 1.1:

- Objective response rate (dose escalation)
- Best overall response
- Duration of response
- Disease control rate
- Time to response
- Progression-free survival

- Pharmacokinetics and immunogenicity of REGN10597

IL, Interleukin; iMAE, immune-mediated adverse event; TEAE, treatment-emergent adverse event.

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